

**BUSINESS ASSETS**

# VYTALIX — Corporate Presentation (12 slides)

Vytalix company-build documentation · reference  
CORPORATE\_PRESENTATION

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Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS\_BASE.

**PREPARED FOR**

The Founder, Vytalix

**DATE**

8 September 2026

**VERSION**

1.0 · Confidential draft

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**Audience:** any first meeting where Vytalix must be introduced as a group (clients, partners, advisers, investors before a dedicated deck). **Length:** 12 slides, 15 minutes. **Version:** 1.0 · 8 September 2026 · Status: PROPOSED.

Design per `/docs/03_BRAND_SYSTEM.md` §13 (16:9, Bone body slides, Deep title, one accent). Language per `/FACTS_BASE.md` §4. Every slide carries the footer "Vytalix — Technology for Life" and a status tag ("Draft — pre-seed").

## Executive view

1. This deck introduces the group, not a product: four pillars, how they connect, what is true today and what is a plan.
2. Its job is to earn a second conversation with a stage-honest story; it makes no claim of clients, partners, approvals or results.
3. MaternaLink appears on two slides only, always as "in development", with its intended use and its gates stated explicitly.
4. Every number is sourced or labelled ESTIMATE; the founder fills the bracketed fields before each use and removes any slide whose claim is not yet true.
5. Derivative decks — `/assets/SALES_DECK.md` (services prospects) and `/assets/PARTNERSHIP_DECK.md` (MaternaLink pilot partners) — reuse slides 2, 3, 4 and 11 from this master.

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## Slide 1 — Title

### Copy

- VYTALIX
- Technology for Life
- A UK health-technology group: advice, delivery, product, learning.
- [Date] · [Audience or meeting name] · Confidential — draft

**Speaker note:** thirty seconds. "Vytalix is a new, founder-led health-technology group. I'll show you what we do, how the four parts fit, and what we're looking for from this conversation."

**Visual:** Deep background; wordmark reverse top-left; Fraunces 72px title in Bone; subtitle in Signal Mint; confluence illustration right third; footer line in Night Muted.

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## Slide 2 — The gap we exist to close

### Copy

- Care fails most often between people.
- Between a woman and a changing set of professionals. Between one shift and the next. Between a community clinic and a hospital. Between a good idea and a safe, adopted product.
- Public inquiries in UK maternity care (Ockenden 2022; Kirkup 2022) repeatedly found failures to listen, escalate and hand over.
- Health-tech founders lose months to assurance and procurement they discover too late.

- Source line: reports named above, gov.uk publications; cite the current MBRRACE-UK report before using any disparity statistic.

**Speaker note:** name the problem plainly; do not quote a mortality statistic unless the current MBRRACE-UK citation is on the slide.

**Visual:** Bone background; eyebrow "THE GAP" in Teal; two-column body; a simple monoline diagram of four "between" gaps as breaks in a path, with the Ember node marking the break.

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## Slide 3 — Who we are

### Copy

- Vytalix is a UK health-technology group that advises, builds, delivers and teaches — so that technology earns its place in care.
- Founder-led. Pre-seed. UK home market, with active interest in maternal health in Nigeria and one further African market.
- Four pillars, one company: Advisory · Consulting · Health Solutions · E-Learning.
- What we are not: NHS-approved, clinically validated, funded, or a team of twenty. Yet.

**Speaker note:** the stage disclosure is a feature. Say it before anyone asks.

**Visual:** Bone; four pillar icons from the brand icon set (compass, spanner-and-path, Y-node shield, open book with signal line) in a row, each labelled; Y-node mark bottom-right.

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## Slide 4 — Four pillars, one loop

### Copy

- Advisory informs Delivery. Delivery informs Product. Product informs Learning. Learning feeds Advisory.
- Advisory: independent, senior advice on strategy, market entry, regulation, funding readiness.
- Consulting: fixed-scope, fixed-price sprints — discovery, business cases, readiness reviews, market-entry playbooks, evaluation.
- Health Solutions: products designed for real care settings, starting with MaternaLink (in development).
- E-Learning: practical courses for health teams and founders (first courses month 9, ESTIMATE; no accreditation claimed).

**Speaker note:** the loop is the point of a group rather than a boutique or a start-up: every engagement produces a reusable asset; every product decision is tested in advisory work first.

**Visual:** a circular four-node monoline diagram, Teal strokes on Bone, Ember node at the centre labelled "care"; pillar labels in Manrope 700.

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## Slide 5 — What we can do for you today

### Copy

- Advisory days and monthly retainers (from £1,200/day; £4,000/month for two days — ESTIMATE).

- Ten fixed-price consulting packages, £9,500–£28,000, two to six weeks each (ESTIMATE).
- Examples: Digital Health Discovery Sprint (2 weeks) · DTAC & Evidence Readiness Sprint (3 weeks) · Digital Maternity Readiness Review (4 weeks) · Health Inequalities & Maternity Outcomes Review (5 weeks).
- Everything founder-led; associates only on signed work; no patient data processed in advisory or consulting.
- Full catalogue: [/assets/SERVICE\\_CATALOGUE.md](#).

**Speaker note:** this is the cash layer and the credibility layer. Say which two packages fit the person in the room and stop.

**Visual:** Bone; three cards (Advisory / Consulting / Workshops) — cards are used because the viewer chooses between siblings (brand rule 6); prices in Manrope 700 with "ESTIMATE, ex-VAT" in Slate.

## Slide 6 — How we work

### Copy

- Fixed scope, fixed price, written change control.
- Evidence before claims: we cite the source or we don't use the number.
- Safety before speed: intended use, clinical safety (DCB0129/0160) and data protection come before features.
- Build with, not for: discovery interviews before design; "built with clinicians" only once true.
- We will tell you if your idea should not be built.

**Speaker note:** these are commitments a buyer can hold us to; they are also the reason we can work in maternity care at all.

**Visual:** five short lines with monoline icons; generous whitespace; no imagery.

## Slide 7 — MaternaLink (in development)

### Copy

- A care-coordination and communication platform for maternity services, designed to help women stay connected to their care team and help care teams hand over safely — for NHS and low-connectivity settings.
- v1 (PROPOSED): enrolment and consent · contact schedules and reminders · signed-off education in the woman's language · secure two-way messaging with a named human owner · a diary displayed, not interpreted · care-team console · structured handover summary · escalation log · missed-contact lists · programme dashboard.
- It does not diagnose, predict, triage or score. It supports — not replaces — clinical judgement.
- Status: early prototype; IP being documented; no data, validation, clinical partner, approval or deployment.

**Speaker note:** read the "does not" line aloud. If the audience wants the AI story, go to slide 8; do not improvise.

**Visual:** Night background section-style slide; product name with the "(in development)" tag rendered as part of the lockup; feature list in two columns; a persistent tag "Non-diagnostic — in development" top-right in Ember Light.

## Slide 8 — Roadmap with gates, not promises

### Copy

- Gates before anything is sold: (1) IP ownership documented · (2) prototype audited for code ownership, security and data · (3) Clinical Safety Officer appointed and hazard log opened · (4) DPIA and lawful-basis analysis complete.
- v1 (12–16 weeks from funded start, ESTIMATE): non-diagnostic MVP; UK service evaluation; Nigeria SMS cohort with an implementing partner.
- v2 (months 7–15, PROPOSED): integration (FHIR, DHIS2 export), WhatsApp/USSD/voice, partner access; any monitoring feature requires a documented device-qualification decision first.
- v3 research track (month 15+, not a product commitment): the 2025 multimodal AI concept, advanced only with data partnerships, ethics approval, validation and a UK MDR / MHRA pathway.
- Detail: [/docs/04\\_MATERNALINK\\_PRODUCT\\_STRATEGY.md](/docs/04_MATERNALINK_PRODUCT_STRATEGY.md) , [/product/MATERNALINK\\_ROADMAP.md](/product/MATERNALINK_ROADMAP.md) .

**Speaker note:** gates are the differentiator. Trusts and ministries have been burnt by unvalidated apps; artefacts beat adjectives.

**Visual:** horizontal waypoint diagram; four gate markers in Ember before the v1 waypoint; v3 drawn as a dotted branch labelled "research"; source line at 16px.

## Slide 9 — Where we work

### Copy

- United Kingdom: home and credibility market — services now; MaternaLink design partnership from month 6–18; paid pilots months 18–36 ( [/docs/05\\_MARKET\\_STRATEGY.md](/docs/05_MARKET_STRATEGY.md) §8).
- Nigeria: volume and impact market — one live conversation (status unknown); the play is partner, complement or move state, never displace on price; NGO- and donor-funded cohorts preferred until willingness to pay is validated.
- Kenya: third market for 2028+, positioned on facility coordination and referral.
- Everywhere else: consulting and e-learning only until 2028.

**Speaker note:** two active product markets before 2028, no more; each extra market costs ~20 unbilled founder days a year (ESTIMATE).

**Visual:** simple three-tier list with a small stylised map made of monoline contour fields (no national flags, no pins over cities).

## Slide 10 — Governance and safety posture

### Copy

- One English company (recommended "Vytalix Group Ltd", name TO VALIDATE) with the four pillars as divisions; founder holds 100% at incorporation; all IP assigned to the company.
- Board: founder plus one independent non-executive (PROPOSED). Advisory board of six profiles being recruited: obstetrics, midwifery/digital midwife, NHS digital and commissioning, Nigerian health-system

leadership, regulatory and clinical safety, health-tech scaling. None appointed yet (KNOWN).

- Policies at incorporation: conflicts of interest, anti-bribery, data protection, clinical-claims sign-off.
- Insurance before the first paid engagement: professional indemnity and public liability (ESTIMATE ranges in `/docs/15_LEGAL_AND_REGULATORY.md` §11).

**Speaker note:** governance is a credibility asset at this stage; say what exists and what is being recruited, and never name an adviser without written consent.

**Visual:** two-column: "In place / In progress" — with dates in the bracketed fields [●].

## Slide 11 — The team

### Copy

- [Founder name], Founder — [two-line background: KNOWN facts only, confirmed against CV].
- Associates: engaged on signed work; named only with consent.
- Advisers: [name, role — only with written consent; otherwise "Recruiting: see slide 10"].
- Declared interests: the founder's separate venture, Kemek Enterprise Ltd (property finance), is declared and unrelated.

**Speaker note:** no invented people. If the slide has one name, it has one name.

**Visual:** one portrait panel (illustration placeholder until real photography is commissioned with consent); roles in Manrope; no stock imagery.

## Slide 12 — What we are looking for / next step

### Copy

- Clients: a first conversation about one of the packages on slide 5 — a 30-minute discovery call, no charge.
- Partners: one UK maternity service to co-design MaternaLink (in development) as an unpaid service evaluation; one Nigerian implementing partner with an existing cohort.
- Advisers: the six profiles on slide 10.
- Investors: not raising today; building in public; SEIS/EIS advance assurance planned after incorporation (TO VALIDATE eligibility).
- Contact: [email] · [LinkedIn] · [website]

**Speaker note:** ask for one specific next step and a date. Then stop talking.

**Visual:** Deep background; wordmark; four short lines; Ember CTA button style for the contact line; footer "Technology for Life".

## Appendix slides (optional, not counted)

- A1 Service catalogue table (from `/assets/SERVICE_CATALOGUE.md` §3).

- A2 MaternaLink intended-use statement (verbatim from `/docs/04_MATERNALINK_PRODUCT_STRATEGY.md` §21.1).
  - A3 "What we don't know yet" — the honesty slide: name clearance, IP agreement, buyer price acceptance, design-partner interest.
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## Priorities / Risks / Next actions

### Priorities

1. Build the slide master per `/docs/03_BRAND_SYSTEM.md` §13 (currently TO DO in the brand checklist) and lay this deck into it.
2. Fill slide 11 with KNOWN facts only and confirm the founder's background line against the CV.
3. Keep slides 7–8 word-for-word aligned with the intended-use statement; any change requires clinical-adviser sign-off ( `/docs/02_GROUP_STRUCTURE.md` §4.3).

### Risks

- A presenter ad-libs "our platform predicts..."; mitigation: speaker notes carry the "does not" line and the deck is the only MaternaLink material shown externally (the 2025 deck is retired).
- Name change after clearance; mitigation: wordmark is a single asset swap.
- Outdated status fields; mitigation: date on every export; monthly review in the founder operating cadence.

### Next actions

- Founder, by Day 14: slide master built; deck v1.0 exported to PDF; appendix A2 pasted verbatim.
- Founder, by Day 30: first three uses logged in the CRM with audience feedback; revise slides 5 and 12 from what buyers asked.
- Founder, by Day 60: slide 10 updated with any adviser appointed (with consent) and insurance in place.